INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Carboplatin + Etoposide

Understand your treatment. Know what to expect. Keep your plan to hand.

**Displayed calendar**
**21 days**
See the active treatment phase

**Treatment stages**
**1**
Explained in the treatment reference

**Regimen source**
**NCCP 00271**
Version 6

Carboplatin on Day 1 and etoposide on Days 1–3, every 21 days. The intended course is usually four to six cycles and may change after review.

## Treatment stages

01 **Day 1 platinum and etoposide; Days 2–3 etoposide**
Three treatment days, then recovery

Your team confirms the active stage and dates. The full treatment guide explains each step.

## Inside your handbook


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**My name** _____

**Hospital / oncology team** _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

# Understanding your treatment

Carboplatin + Etoposide · National Cancer Control Programme (NCCP) 00271

## When to get help

**Emergency: call 112 / 999**

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

**Contact your oncology service immediately**

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call urgently for fever or shaking chills, sudden illness, new or worsening breathlessness, chest pain, uncontrolled bleeding, severe diarrhoea, vomiting despite medicines, inability to drink, very little urine, or a spreading/blistering rash. Tell staff immediately about wheeze, swelling, dizziness or faintness during an infusion. Report hearing changes, ringing in the ears, numbness, weakness or difficulty walking promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

## Your two chemotherapy medicines

Carboplatin + Etoposide is chemotherapy used for extensive-stage small-cell lung cancer. It aims to control the cancer and its symptoms. Ask your oncology team what benefit is realistic for you and how this option compares with other suitable treatments.

PLATINUM CHEMOTHERAPY

### Carboplatin



**Forms links that damage deoxyribonucleic acid (the cell's genetic material) (DNA)**

Carboplatin forms links in DNA that interfere with copying and repair. This helps stop cancer cells dividing; normal tissues can also be affected. Carboplatin dosing is tailored using a verified kidney-function result; low blood counts can be an important limiting effect.

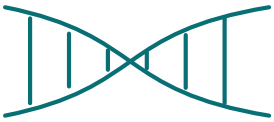
Platinum enters cells → DNA links form → DNA copying disrupted

**What to watch for**

Report changes in urine, hearing, ringing in the ears, numbness or daily function. Tell the team about infection, unusual bruising or bleeding and any symptoms during the infusion.

TOPOISOMERASE II CHEMOTHERAPY

### Etoposide



**Disrupts DNA repair during cell division**

Etoposide interferes with topoisomerase II, an enzyme that helps manage DNA during copying. DNA breaks can remain unrepaired and cancer cells may stop dividing or die. Healthy cells are also affected; the infusion or prescribed capsules deliver the same medicine by different routes.

Topoisomerase II disrupted → DNA breaks persist → Cell division interrupted

**What to watch for**

Report fever, bleeding, mouth sores or diarrhoea. Tell the nurse about faintness, rash or breathing difficulty during an infusion. If capsules are prescribed, use the exact pharmacy instructions and contact the team about a missed or vomited dose.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

## Evidence and your own treatment decision

The exact medicines, dose, population and treatment era matter.

Platinum and etoposide are established chemotherapy components for extensive-stage small-cell lung cancer (SCLC). A pooled comparison found no clear survival difference between carboplatin-based and cisplatin-based treatment, but their side effects differed. The elderly/poor-risk JCOG9702 trial used etoposide 80 mg/m² and split cisplatin, so its survival figures cannot be assigned directly to this 100 mg/m² etoposide schedule. These older chemotherapy comparisons do not rank current immunotherapy combinations or predict an individual outcome.

SACTCHECK · CHAPTER 02

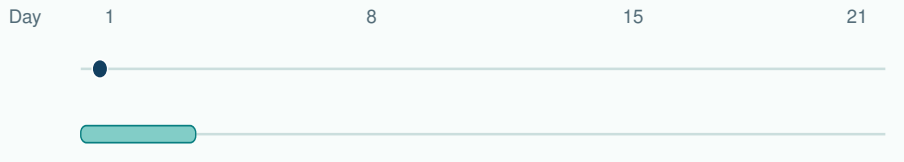
# Your schedule, tests and appointments

Carboplatin + Etoposide · NCCP 00271

## Three treatment days in a 21-day cycle

**Carboplatin + Etoposide**

21-day view




This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

THREE TREATMENT DAYS, THEN RECOVERY

### Day 1 platinum and etoposide; Days 2–3 etoposide

D1 D2–D3 D4–D5 D6–D21

Carboplatin is planned on Day 1. Etoposide is planned on Days 1–3: usually all three days by infusion, or a prescribed alternative with an infusion on Day 1 and capsules twice daily on Days 2–3. The source describes 4–6 cycles every 21 days, or stopping for cancer progression or unacceptable side effects. Your team confirms the intended course and may change it earlier.

The source describes 4–6 cycles every 21 days, or stopping for cancer progression or unacceptable side effects. Your team confirms the intended course and may change it earlier.

BASELINE → TREATMENT → FOLLOW-UP

## Checks through the whole course

The columns show the order of care, not equal time intervals. Extra tests depend on symptoms, results and the consultant's plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00271 v6 · official regimen · NCCP NCIS Medical Oncology antiemetics V8, Table 1 and multi-day guidance · Manufacturer carboplatin product information

THE WHOLE TREATMENT JOURNEY

## Carboplatin + Etoposide: treatment and checks

### Extensive-stage small-cell lung cancer · chemotherapy

Carboplatin is planned on Day 1. Etoposide is planned on Days 1–3: usually all three days by infusion, or a prescribed alternative with an infusion on Day 1 and capsules twice daily on Days 2–3. The source describes 4–6 cycles every 21 days, or stopping for cancer progression or unacceptable side effects. Your team confirms the intended course and may change it earlier.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Days 1–3 of each cycle | Between visits | Course / response review | After chemotherapy |
|---|---|---|---|---|---|
| **Chemotherapy course**<br>NCCP 00271 | Agree indication, route and course | Day 1 Carboplatin + etoposide; etoposide continues Days 2–3 | Recovery and symptom monitoring | The source describes 4–6 cycles every 21 days, or stopping for cancer progression or unacceptable side effects. Your team confirms the intended course and may change it earlier. | Follow-up or a separately prescribed next plan |
|  <br>**Blood counts**<br>NCCP 00271 | Baseline blood count | Review results before treatment | Blood count weekly before treatment | Review infection, bleeding and previous toxicity | Follow unresolved low counts |
| **Kidneys, liver and fluids**<br>NCCP 00271 | Renal/liver profile; verify clearance and dose suitability | Renal/liver tests before each cycle; verify current carboplatin GFR | Repeat sooner after dehydration or illness | Review organ function before continuation or modification | Follow kidney/ electrolyte/ liver abnormalities |
| **Carboplatin dose verification**<br>NCCP 00271 | Measured GFR preferred or validated estimate | If estimated: creatinine within 48 hours of dosing | Reassess if health or kidney function changes | If creatinine >110% of isotope-baseline value, reassess GFR | A future regimen requires its own dose review |
| **Hearing and nerves**<br>NCCP 00271 | Record hearing/nerve symptoms and previous platinum exposure | Regular hearing and neurological review | Report tinnitus, hearing loss, numbness or loss of function | Balance benefit and lasting toxicity | Review persistent changes |
| **Prevention and route**<br>NCCP 00271 · ANTIEMETIC | Confirm infusion or capsule plan and a written multi-day anti-sickness schedule | Check support before each treatment day; pharmacy confirms oral doses | Follow support through two days after last systemic anticancer therapy (SACT); do not extend medicines yourself | Review interactions and support after any dose/ route change | Confirm which support medicines stop or continue |
|  <br>**Disease assessment**<br>NCCP 00271 | Confirm extensive-stage SCLC and individual treatment aim | Assess symptoms and fitness | Extra assessment for new concerns | Scans/tests as directed by consultant; no universal scan interval | Agree follow-up or a separate next treatment plan |

### What each check is for

1

#### Before first treatment

Before dose authorisation

Confirm suitability and route.

**Ask / act:** Confirm the complete plan with the consultant and pharmacy.

2

#### Across Days 1–3

Every treatment day

The active medicines and route change within each cycle.

**Ask / act:** Confirm dose, route and today's supportive medicines.

3

#### After the last treatment day

Two days after multi-day SACT

Prevention extends beyond the final dose.

**Ask / act:** Provide an individual written schedule and rescue instructions.

4

#### Course and response review

During and at the end of planned chemotherapy

Benefit and toxicity guide the next step.

**Ask / act:** Confirm completion, persistent-toxicity follow-up and any separate next regimen.

#### At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

#### Before each visit

Confirm transport for Days 1–3, blood-test arrangements, prescriptions and the 24-hour contact details. Tell the team if you are ill before travelling.

STEP 2

### Time in the unit

Carboplatin is infused over about 30 minutes, followed by etoposide over about one hour on Day 1. Etoposide alone is given on Days 2 and 3 unless the prescribed capsule alternative is selected. The whole visit also includes review, preparation, prevention medicines and observation.

STEP 3

### Infusion access

Treatment may use a cannula or an existing suitable central line. Ask for line-care instructions if you have a line. There is no routine take-home infusion pump.

STEP 4

### Capsules and home medicines

If prescribed, confirm your etoposide capsule doses, empty-stomach timing, safe handling and what to do if a dose is missed or vomited. Keep prevention and rescue medicines clearly labelled.

STEP 5

### Support between visits

Arrange practical help if needed and keep symptom notes. Bring questions and a current medication list to review.

SACTCHECK · CHAPTER 03

# Symptoms and supportive care

Carboplatin + Etoposide · NCCP 00271

## Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

| Treatment or check | Timing in this view |
|---|---|
| **Carboplatin** |  **Day 1** Carboplatin on Day 1 after clinical review |
| **Etoposide** |  **Days 1–3** Infusion Days 1–3, or prescribed Day 1 infusion / Days 2–3 capsules |
| **Prevention medicines** |  **Days 1–5** Follow the individually reconciled multi-day plan |
| **Report symptoms** |  **Days 1–21** Symptoms may occur at any time; this bar does not measure risk |
| **Next-cycle review** |  **Days 20–21** Tests and review before deciding the next cycle; appointments may differ |

**Before the first dose:** Confirm the diagnosis, treatment aim, blood tests, kidney/liver function and selected etoposide route.

**During the three treatment days:** Day 1 includes both medicines; Days 2–3 continue etoposide by the prescribed route.

**Support after the last dose:** The multi-day sickness-prevention plan needs to account for two days after the last SACT day. Take only the written prescribed course.

**Across and after the course:** Counts, kidneys, hearing and nerves need review throughout treatment. Some problems can persist after the final dose.

**Why these bands stay broad:** This shows one reference cycle and the order of care. It does not predict a symptom-free day or when blood counts will fall. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00271 v6 · official regimen ↗ · NCCP NCIS Medical Oncology antiemetics V8, Table 1 and multi-day guidance ↗

**There are no guaranteed "safe days".** Follow your unit's temperature threshold and emergency instructions. Sudden illness or shaking chills needs urgent advice even without a high temperature.

**Between-treatment notes**

### Day 1: Day 1 in the unit

Carboplatin is infused over about 30 minutes, followed by etoposide over about one hour on Day 1. Etoposide alone is given on Days 2 and 3 unless the prescribed capsule alternative is selected. The whole visit also includes review, preparation, prevention medicines and observation. Tell staff immediately about rash, swelling, wheeze, faintness or pain near the drip.

### Day 2–3: Etoposide continues

Return for etoposide infusions unless the team prescribed the oral alternative. If you have capsules, follow the twice-daily pharmacy label and empty-stomach instructions. Confirm today's anti-sickness medicines. Contact the team about a missed or vomited capsule dose.

### Day 4–5: After the last treatment day

Follow the written prevention and rescue plan. Multi-day antiemetic cover extends to two days after the final treatment day; the exact medicines are chosen by your team. Call for sickness despite medicines, inability to drink or reduced urine.

### Day 6–20: Recovery and symptom monitoring

Fatigue, low counts, infection, mouth or bowel symptoms can occur between visits. Hearing and nerve symptoms may build across cycles. Use the urgent contact plan at any time; do not wait for the next appointment.

### Day 21: Review before the next cycle

The team reviews blood counts, kidney/liver function and symptoms before deciding to continue, delay or change the plan. Bring your medicine list and symptom notes; confirm the remaining course and next review.

## Support around the medicines

### 1 Preventing sickness across all treatment days

Take the written anti-sickness plan even if you feel well. The plan must cover the risk of the medicines on each of Days 1–3 and the two days after the last treatment. Ask the pharmacist to show which medicines are scheduled and which are rescue medicines.

### 2 One clear steroid plan

The anti-sickness plan may include dexamethasone. Follow the prescribed day-by-day instructions; do not add an extra steroid course or extend it yourself. Tell the team about diabetes, poor sleep or mood changes.

### 3 Fluids and kidney protection

Ask for a personal drinking plan. Kidney results help calculate the carboplatin dose; illness or dehydration may require repeat tests. Call if you cannot drink or pass much urine.

### 4 If etoposide capsules are prescribed

The source alternative uses capsules on an empty stomach twice daily on Days 2 and 3. Your pharmacy label gives your individual capsule dose. Do not substitute capsules for an infusion yourself, crush/open capsules, double a missed dose or repeat a vomited dose without advice. Obtain safe handling and storage instructions.

### 5 Medicines, mouth care and infection advice

Check prescriptions, painkillers, supplements and vaccines with the oncology team. Some medicines affect kidney/hearing toxicity or etoposide exposure. Follow mouth-care advice and keep the unit's urgent contact instructions available.

## Your treatment and its aim

Confirm the full plan before starting.

### The treatment covered here

This summary is for chemotherapy for extensive-stage small-cell lung cancer. Treatment that also includes immunotherapy or radiotherapy needs its own complete plan.

### Why two medicines?

The platinum drug damages DNA and etoposide interferes with DNA repair. Using them together attacks cell growth in different ways. Healthy cells can also be affected.

## Before treatment

Preparation helps the team choose a suitable plan.

### Tests and fitness

Blood counts, kidney and liver tests are checked before treatment. Tell the team about existing nerve/hearing problems, other medicines and any recent infection.

### Route and practical arrangements

Confirm whether all three etoposide doses are infusions or whether you have capsules on Days 2 and 3. Do not substitute one route yourself.

### Fertility, contraception and pregnancy

Discuss fertility preservation and contraception before treatment. Pregnancy and breastfeeding are exclusions. Ask for medicine-specific advice and the required contraception period.

## Your visits and home plan

Three treatment days form one cycle.

### Days 2 and 3

Continue etoposide by the prescribed route, with risk-appropriate anti-sickness prevention each treatment day.

### The following days

Continue only the written prevention schedule and monitor symptoms. The two days after the final treatment day are part of the multi-day antiemetic review.

## Effects to discuss and monitor

Report problems early; they are not a measure of benefit.

### Blood counts and infection

Both medicines may lower blood counts. Infection can become serious quickly; follow the urgent advice plan for fever, chills or sudden illness.

### Hearing, nerves and kidneys

Platinum medicines may affect hearing, nerves and kidneys, and some changes can last. Tell the team promptly about tinnitus, hearing change, numbness or reduced urine.

### Mouth, bowel, hair and fatigue

Mouth soreness, nausea, bowel changes, hair loss and tiredness can affect daily life. Ask for help with symptoms; call urgently if you cannot drink or symptoms are severe.

### Later effects

Etoposide can be associated with a later blood cancer. Discuss serious and lasting risks together with likely benefit and alternatives.

## When to seek urgent help

Use your unit’s 24-hour number and emergency instructions.

**When calling**

Give your regimen, last treatment day and symptoms. Explain whether you can drink and pass urine. Do not wait for the next appointment or for a high temperature if you feel very unwell.

## Review and the next plan

Completion and follow-up depend on the treatment aim and response.

**Course review**

**Disease assessment**

Scans and other assessments are arranged by the consultant. Symptoms alone do not show whether treatment is working.

**After the last dose**

Review ongoing kidney, hearing, nerve and blood-count problems. Confirm which support medicines finish and agree follow-up. Any next treatment, including immunotherapy or radiotherapy, needs a separate plan.

## Questions worth asking

**Why are there three treatment days?**

Etoposide is planned on three consecutive days. Usually you attend for infusions on all three. The alternative capsule plan is only used when prescribed and clearly labelled by your team.

**Are etoposide capsules the same dose as the drip?**

No. The source oral alternative has a different dose and twice-daily schedule. Your prescription and pharmacy label specify your individual dose and capsule strengths. Do not convert it yourself.

**Will I have radiotherapy or immunotherapy too?**

This guide covers Carboplatin + Etoposide chemotherapy for extensive-stage small-cell lung cancer. Immunotherapy combinations and limited-stage treatment with radiotherapy use separate regimens. If an additional medicine or radiotherapy is prescribed, obtain its own plan and consent information.

**How long does the course last?**

The source describes 4–6 cycles every 21 days, or stopping for cancer progression or unacceptable side effects. Your team confirms the intended course and may change it earlier. The passport pages are reusable records, not a prescription for a fixed number of visits.

**Do more side effects mean treatment is working?**

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be assessed and treated; your team reviews benefit with the agreed clinical and scan plan.

### Why might treatment be delayed or changed?

Blood counts, kidney/liver function or symptoms may need time to recover. A dose may change to reduce lasting harm. Ask which medicine changes, why, and when it will be reviewed.

### What should I say when I call?

Give the regimen name, last treatment day, symptoms and when they started. Explain whether you can drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect information.

### Can this guide decide whether treatment is safe?

No. Treatment decisions need your clinical results, symptoms, previous toxicity and prescriber/pharmacy review. This guide helps you understand and discuss the plan.

### What should I be able to explain before I leave?

Ask your nurse or doctor to check that you can explain which medicines you are receiving, the treatment days, which medicines to take at home, and which number to ring urgently. Ask for the plan in writing if any part is unclear.

> **Try explaining the plan back:** Can you describe which days you have treatment, whether you have etoposide capsules, how you take prevention medicines and which number to call at any hour?

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**Which treatment this guide covers** This guide covers Carboplatin + Etoposide chemotherapy for extensive-stage small-cell lung cancer. Immunotherapy combinations and limited-stage treatment with radiotherapy use separate regimens. If an additional medicine or radiotherapy is prescribed, obtain its own plan and consent information.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Carboplatin + Etoposide · standard three-day course

**Active medicines:** carboplatin + etoposide

**Emetogenic risk:** high

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally Day 1; 80 mg orally Days 2–3 | NCIS high-risk reference course for Day 1 platinum<br>Reconcile the full multi-day course; the special extension after five-day cisplatin is not automatically applied to this three-day etoposide course. |
| Ondansetron | 16 mg orally Day 1 (high-risk reference) | Before Day 1 treatment<br>NCIS low-risk etoposide-day reference is ondansetron 8 mg orally once daily or a corticosteroid. Days 2–3 prevention must be reconciled with the existing dexamethasone course; do not duplicate agents automatically. Adjust for QT/ individual risk. |

| Medicine | Dose / route | When |
|---|---|---|
| Dexamethasone | 12 mg orally Day 1; 8 mg orally daily Days 2–4 (high-risk reference) | A component of the individual Days 1–5 plan<br>These are the NCIS high-risk row doses. Pharmacy must reconcile overlapping delayed-platinum and etoposide-day prevention, and specify the Day 5 plan; do not invent a uniform steroid extension. |
| Metoclopramide | 10 mg orally three times daily as prescribed | Subsequent days / rescue instructions set by prescriber<br>High-risk NCIS row has no end day or PRN qualifier. The low-risk row uses PRN. Specify role/duration and check suitability; avoid unreviewed duplication. |

## Additional phase-specific support

- NCIS V8 p 5: provide antiemetics appropriate to the risk of systemic anticancer therapy (SACT) on each treatment day and for two days after; for standard Days 1–3 treatment, review coverage through Day 5.
- Day 1 high-risk platinum and Days 2–3 low-risk etoposide are different exposures. Table 1 alone does not give one universal three-day platinum/etoposide prescription.
- Do not invent extra steroid days or extend aprepitant using the separate five-day cisplatin exception.
- Confirm one written steroid/antiemetic course, QT risk, interactions and rescue medicines.
- intravenous (IV) and oral etoposide alternatives require matching pharmacy and patient instructions.
- Use dedicated carboplatin AUC≥4 NCIS row, not routine olanzapine from the generic high-risk row.
- NCCP 00271 gives no additional prescribed hydration or supportive-care schedule; use individually agreed unit policy.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Ears / hearing

**Carboplatin**

Report tinnitus or hearing change promptly; discuss existing problems before starting.

### 3 Chest / breathing

**Both medicines**

New breathlessness, wheeze or chest pain needs urgent assessment.

### 5 Stomach / bowel

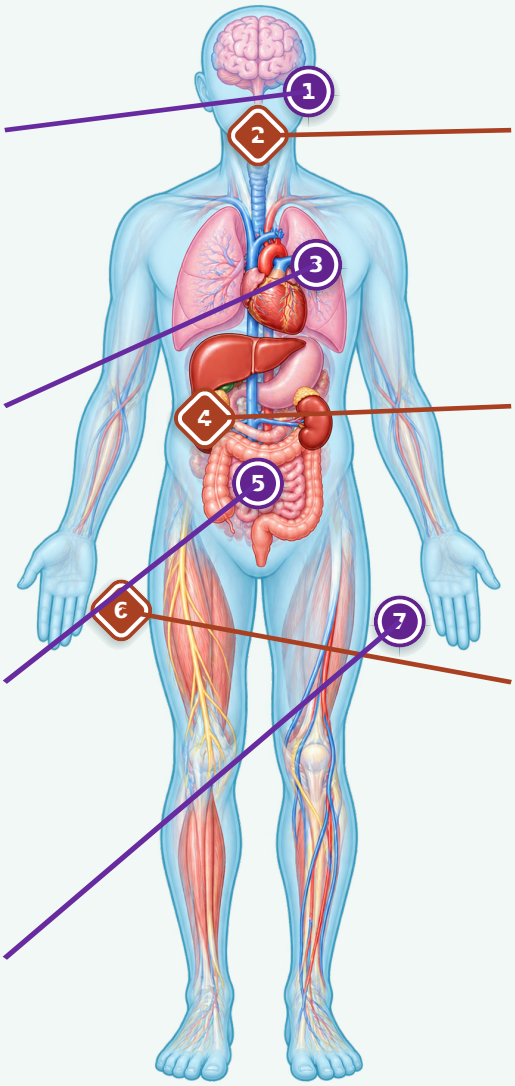
**Both medicines**

Use prescribed anti-sickness medicines; severe vomiting or diarrhoea needs advice.

### 7 Skin / blood counts

**Both medicines**

Report rash, unusual bruising or bleeding. Fever or sudden illness needs urgent advice.




### 2 Mouth

**Etoposide**

Mouth sores can affect eating or drinking. Seek advice early.

### 4 Kidneys / fluids

**Carboplatin**

Report reduced urine or inability to drink; kidney tests guide treatment.

### 6 Hands / feet

**Carboplatin**

Report numbness, tingling or problems with buttons, balance or walking.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call urgently for fever or shaking chills, sudden illness, new or worsening breathlessness, chest pain, uncontrolled bleeding, severe diarrhoea, vomiting despite medicines, inability to drink, very little urine, or a spreading/blistering rash. Tell staff immediately about wheeze, swelling, dizziness or faintness during an infusion. Report hearing changes, ringing in the ears, numbness, weakness or difficulty walking promptly.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

SACTCHECK · CHAPTER 04

# Sources and further information

Carboplatin + Etoposide · National Cancer Control Programme (NCCP) 00271

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **National Cancer Control Programme (NCCP) 00271 v6 · official regimen**
   https://healthservice.hse.ie/documents/6931/271_CarboplatinAUC5Etop100.pdf
2. **NCCP NCIS Medical Oncology antiemetics V8, Table 1 and multi-day guidance ↗**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
3. **Manufacturer carboplatin product information**
   https://www.medicines.org.uk/emc/product/3787/smpc
4. **Manufacturer etoposide product information: mechanism, risks and precautions**
   https://www.medac.eu/fileadmin/user_upload/medac-eu/SPCs/United_Kingdom/Etoposide-spc-gb.pdf
5. **HSE chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
6. **COCIS: individual-patient meta-analysis of first-line platinum chemotherapy in small-cell lung cancer (SCLC)**
   https://pubmed.ncbi.nlm.nih.gov/22473169/
7. **JCOG9702: carboplatin/etoposide versus split cisplatin/etoposide in elderly or poor-risk extensive SCLC**
   https://www.nature.com/articles/6603810
8. **HPRA Vepesid capsules: patient leaflet**
   https://assets.hpra.ie/products/Human/30581/94054f4b-91cf-4c49-8063-040ead5c4a92.pdf